

ECORT CREAM

MAL19913384A

DESCRIPTION:

CREAM

Colour : White
Fragrance : Natural

CONTENT:

Econazole Nitrate1% w/w
Triamcinolone Acetonide..... 0.1% w/w
Preservative: Chlorocresol 0.2% w/w

PHARMACODYNAMICS:

Econazole is an imidazole antifungal agent and may act by interfering with the permeability of the fungal cell membrane. It has a wide antifungal spectrum and possesses some antibacterial activity.

Triamcinolone Acetonide is an adrenocorticoid which when applied externally will produce anti-inflammatory and anti-allergic reactions in the skin. It diffuses across cell membranes and complexes with specific cytoplasmic receptors. These complexes then enter the cell nucleus, bind to DNA (chromatin), and stimulate transcription of messenger RNA (mRNA) and subsequent protein synthesis of various enzymes thought to be ultimately responsible for anti-inflammatory effects of topical application of adrenocorticoids.

PHARMACOKINETICS:

When applied externally, there is very little absorption of Econazole Nitrate through skin or mucous membranes.

When applied externally, Triamcinolone Acetonide can be absorbed systematically and occlusion or use over extensive areas increases absorption of topical applied drug. Metabolism is mostly in the skin tissue.

INDICATIONS:

For the topical treatment of inflammatory dermatomycoses and inflammatory skin conditions complicated by or threatened by bacterial or fungal skin infection.

RECOMMENDED DOSE:

Apply a thin film of the cream to the affected area twice daily for 14 days.

Following disappearance of the inflammation, consideration should be given to continuing treatment with anti-fungal therapy alone until two weeks after clinical cure have been achieved.

CONTRAINDICATIONS:

History of sensitivity reaction to Triamcinolone or other topical corticosteroids or Econazole or other imidazole antifungal agents. Tuberculosis of the skin, and viral diseases, e.g. herpes simplex, varicella, vaccinia, acne vulgaris. It should not be used in pregnant patients in large amounts or for prolonged period of time.

WARNING AND PRECAUTIONS:

Treatment should be discontinued if irritation or sensitization develops with the use of the cream and appropriate therapy instituted. In the presence of an infection, the use of an appropriate antifungal or antibacterial agent should be instituted. If a favourable response does not occur promptly, the corticosteroids should be discontinued until the infection is controlled adequately. Any side effects that are reported following systematic use of corticosteroids, including adrenal suppression, may also occur with topical corticosteroids, especially in infants and children. Systematic absorption of topical corticosteroids will be increased if extensive body surface areas are treated or if the occlusive technique is used. Suitable precautions should be taken under these conditions or when long-term use is anticipated.

Do not use in children under 2 years of age except under the advice and supervision of a doctor.

If irritation occurs or if there is no improvement within 4 weeks, discontinue use and consult a doctor or pharmacist.

PREGNANCY AND LACTATION:

Topical administration of corticosteroids to pregnant animals can cause abnormalities of foetal development. The relevance of this finding to human beings has not been established. However, topical steroids should not be used extensively in pregnancy, ie in large amounts or for prolonged periods.

SIDE EFFECTS/ADVERSE REACTIONS:

Local adverse reactions reported with the use of topical corticosteroids, especially under occlusive dressings, include burning, itching, irritation, dryness, folliculitis, hypertrichosis, acneiform eruptions, hypopigmentation, perioral dermatitis, allergic contact dermatitis, maceration of the skin, secondary infection, skin atrophy, striae and miliaria.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Symptoms: Excessive prolonged use of topical corticosteroids can suppress pituitary-adrenal function resulting in secondary adrenal insufficiency.

Treatment: Appropriate symptomatic treatment is indicated. Acute hypercorticotoid symptoms are virtually reversible.

Treat electrolyte imbalance, if necessary. In cases of chronic toxicity, slow withdrawal of steroids is advised.

PACKING/PACK SIZE(S):

Aluminium tube of 15 g and 50x15 g.

**JAUHI UBAT DARIPADA KANAK-KANAK
KEEP OUT OF REACH OF CHILDREN
FOR EXTERNAL USE ONLY**

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

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